Despite This Realization, NHP-2017 Essentially Remains a Public-Sector Health Policy

NHP-2017 sets its goals in most ambitious, and general, terms. It envisages the ‘attainment of the highest possible level of health and well-being for all at all ages’ and ‘universal access to good quality healthcare services without anyone having to face financial hardship as a consequence’. That said, the background document made it evident that this was not the reality in the healthcare sector and that the government was in no position to achieve these goals through public-sector expansion of healthcare services.

The Policy also sets specific targets to be achieved by 2025, including life expectancy of 70, reduction of the total fertility rate (TFR) to 2.1; under-five mortality rate to twenty-three; maternal mortality rate (MMR) to 100 (by 2020); and infant mortality rate to twenty-eight (by 2019); as well as increasing the utilization of public health facilities to 50 per cent and the government’s healthcare expenditure to 2.5 per cent of GDP. In addition, it mentions many more disease-specific and health infrastructures.

The largest health policy goal is to raise public health expenditure to 2.5 per cent of the GDP ‘in a timebound manner’ and allocate resources to the states linked to state development indicators, absorptive capacity and financial indicators. According to NHP-2017, the increased expenditure will be financed by ‘general taxation’ and the government would consider imposing taxes on specific commodities like tobacco, alcohol, junk food, the extractive industry and pollution.

The Policy proposes a significant shift in the organization of public healthcare delivery. Primary healthcare would be substantially expanded with the policy goal being changed from ‘selective care’ to ‘assured comprehensive care with linkages to referral hospitals’. The secondary and tertiary care policy goal would be changed from ‘input-oriented to output-based strategic purchasing’. The strategy of ‘user fees and cost recovery’ is slated to be abandoned in favour of ‘assured free drugs, diagnostic and